

INDIVIDUAL CASE SAFETY REPORT (ICSR)

PROCESSING: CASE 1

Section 1: Literature Intake Sheet

Article ID	IJBCP9(3)
DOI	http://dx.doi.org/10.18203/2319-2003.ijbcp20200731
Journal	International Journal of Basic & Clinical Pharmacology
Year	2020 (March)
Author	Lakshmi Narasimha G., Kalpana P.
Country	India
Patient Age	22 years old
Sex	Male
Drug	Amoxicillin-clavulanic acid (Tablet) and Amoxicillin sodium (Injection)
Reaction	Toxic epidermal necrolysis (TEN)
Serious?	Yes
Reporter Type	Healthcare Professional (Pharmacist/Dermatologist)
Source	Literature Case Report

Section 2: Case Triage

Patient?	Yes (22-year-old male)
Reporter?	Yes (Department of Pharmacy Practice / Dermatologist)
Drug?	Yes (Amoxicillin-clavulanic acid and Amoxicillin sodium)
Drug Reaction?	Yes (Toxic epidermal necrolysis)

Triage Decision: **VALID ICSR**. All four minimum criteria for an Individual Case Safety Report are present.

Section 3: Data Extraction Sheet

Patient ID	CASE001
Age	22 years
Gender	Male
Weight	Not reported
History	Not reported
Medical History	Fever (onset 3 days prior to admission)
Suspect Drug	Amoxicillin (administered as Amoxicillin-clavulanic acid tablet and Amoxicillin sodium injection)
Dose	625 mg (Tablet) and 250 mg (Injection)
Frequency	Twice daily (for both formulations)
Route	Oral (Tablet) and Intravenous/Intramuscular (Injection)
Lot Number	Not reported

Start Date	3 days prior to admission
Stop Date	Date of hospital admission / diagnosis
Concomitant Drugs	Clavulanic acid (co-formulated with the suspect tablet)
Lab Values	Hb: 10.5 gm%, TC: 1200 cells/cu mm, RBS: 84 gm/dl, BUN: 20 mg/dl, Bicarbonates: 23 mEq/l, Platelets: 2.0 lakhs, Serum glucose: 110 mg/dl. LFTs and RFTs: Normal.
Reaction	Toxic epidermal necrolysis (erythematous macular lesions with darker hemorrhagic patterns, skin sloughing/peeling on the trunk and four limbs, burning sensation, Nikolsky's sign positive)
Outcome	Recovered / Discharged after one week
Action Taken	Drug discontinued
Dechallenge	Positive (Condition significantly improved within one week of drug withdrawal)
Rechallenge	Not performed / Not ethically feasible
Reporter Assessment	Causative agent confirmed as amoxicillin by the attending dermatologist. SCORTEN calculated as 1 (estimated mortality risk of 3.2%).

Section 4: Narrative Writing

Clinical Narrative

Case Summary: A 22-year-old male with no significant past medical history experienced a severe, life-threatening cutaneous reaction following the administration of oral and injectable amoxicillin formulations for a 3-day history of fever.

Onset and Clinical Progression: Within 3 days of initiating the amoxicillin regimen, the patient acutely developed severe skin manifestations, characterized by a generalized burning sensation, extensive skin sloughing, and small blisters distributed across the trunk and all four limbs. Due to the severity of the reaction, the patient was hospitalized at the Government General Hospital, Kadapa.

Physical Examination & Diagnostics:

- **Cutaneous:** Widespread erythematous macular lesions displaying darker hemorrhagic patterns. Active dislodgement of the epidermis was noted on all four limbs.
- **Mucosal/Genital:** Absent (No mucosal or genital involvement noted).
- **Clinical Sign:** Nikolsky's sign was explicitly **positive**.
- **Severity Assessment:** SCORTEN (Score of Toxic Epidermal Necrolysis) was calculated as **1** (based on >10% epidermal detachment), indicating a predicted mortality risk of 3.2%.

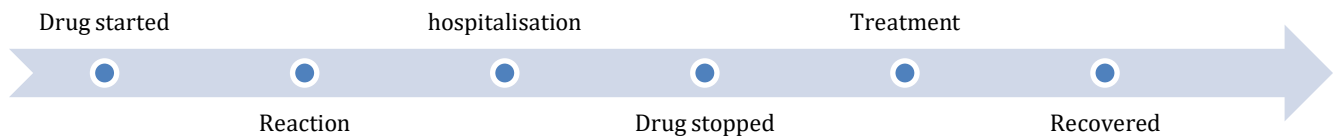
Laboratory Findings:

- **Hematology:** Hemoglobin: 10.5 gm% | Total Count (TC): 1200 cells/cu mm (Leukopenia noted).
- **Biochemistry:** Liver Function Tests (LFTs) and Renal Function Tests (RFTs) were within normal physiological limits.

Management & Outcome: Both formulations of amoxicillin were permanently withdrawn upon admission. The patient was treated with systemic and topical supportive therapy:

- **Systemic:** Injection Dexamethasone (2 cc), Injection Pheniramine maleate (2 cc), and Promethazine (25 mg).
- **Topical:** Silver Sulfadiazine and Fusidic Acid ointments.

Following one week of active in-patient management, the cutaneous manifestations resolved significantly. The patient recovered and was safely discharged.



Section 5: MedDRA Coding

Lowest Level Term (LLT)	Toxic epidermal necrolysis
Preferred Term (PT)	Toxic epidermal necrolysis
High Level Term (HLT)	Bullous conditions
High Level Group Term (HLGT)	Epidermal and dermal conditions
System Organ Class (SOC)	Skin and subcutaneous tissue disorders

*****MedDRA numeric codes omitted because the terminology is licensed; hierarchy shown for educational purposes.***

Section 6: Seriousness

Seriousness: Yes

Reason for Seriousness:

Death	
Life threatening	X
Hospitalisation	X
Congenital anomaly	
Disability	
Other medically important condition	X

Section 7: Expectedness

Classification: Expected

Evidence: According to standard reference safety information (such as the SmPC and USPI for Amoxicillin/Amoxicillin-clavulanic acid), severe cutaneous adverse reactions (SCARs)—specifically including Stevens-Johnson syndrome (SJS) and Toxic Epidermal Necrolysis (TEN)—are known, rare, class-effect hypersensitivity risks associated with beta-lactam antibiotics. Warnings regarding these reactions are standard in product labeling, requiring immediate discontinuation of the medication.

United States Prescribing Information (USPI)

- Warnings and Precautions (Section 5.2).
- Adverse Reactions - Postmarketing Experience (Section 6.2)
- Patient Counseling Information (Section 17)
- Contraindications (Section 4)

Summary of Product Characteristics (SmPC)

- Special Warnings and Precautions for Use (Section 4.4).
- Undesirable Effects (Section 4.8)
- Contraindications (Section 4.3)

Section 8: Causality Assessment

WHO-UMC Causality Category

Category: Probable / Likely

Reason: The adverse event followed a reasonable time sequence after drug intake, followed a clinically plausible recovery pattern upon dechallenge, and cannot be reasonably attributed to the underlying fever or other documented comorbidities.

Naranjo Score Assessment

1. Previous conclusive reports on this reaction?	Yes (+1)
2. Did the adverse event occur after the suspected drug was administered?	Yes (+2)
3. Did the adverse reaction improve when the drug was discontinued?	Yes (+1)
4. Did the adverse reaction reappear when the drug was readministered?	No / Not Done (0)
5. Are there alternative causes that could have on their own caused the reaction?	No (+2)
6. Did the reaction reappear when a placebo was given?	No / Not Done (0)
7. Was the drug detected in the blood in concentrations known to be toxic?	No / Not Done (0)
8. Was the reaction more severe when the dose was increased / less severe when decreased?	No / Not Done (0)
9. Did the patient have a similar reaction to the same/similar drugs in previous exposure?	No / Unknown (0)
10. Was the adverse event confirmed by any objective evidence?	Yes (+1) (Dermatologist evaluation and positive Nikolsky's sign)

Total Naranjo Score: +7

Causality Interpretation: Probable (Score 5–8 indicates a probable relationship)

Section 9: Quality Check

Missing patient info	None
Dates logical	Yes (Onset occurred exactly 3 days post-initiation; recovery occurred 1 week post-discontinuation)
Duplicate events	Checked; no duplicate reporting identified.
Narrative complete	Pass (Reviewer initials:MJ, 10 july,2026)
MedDRA verified	Yes
Seriousness correct	Yes
Causality complete	Yes
CIOMS Form	Complete